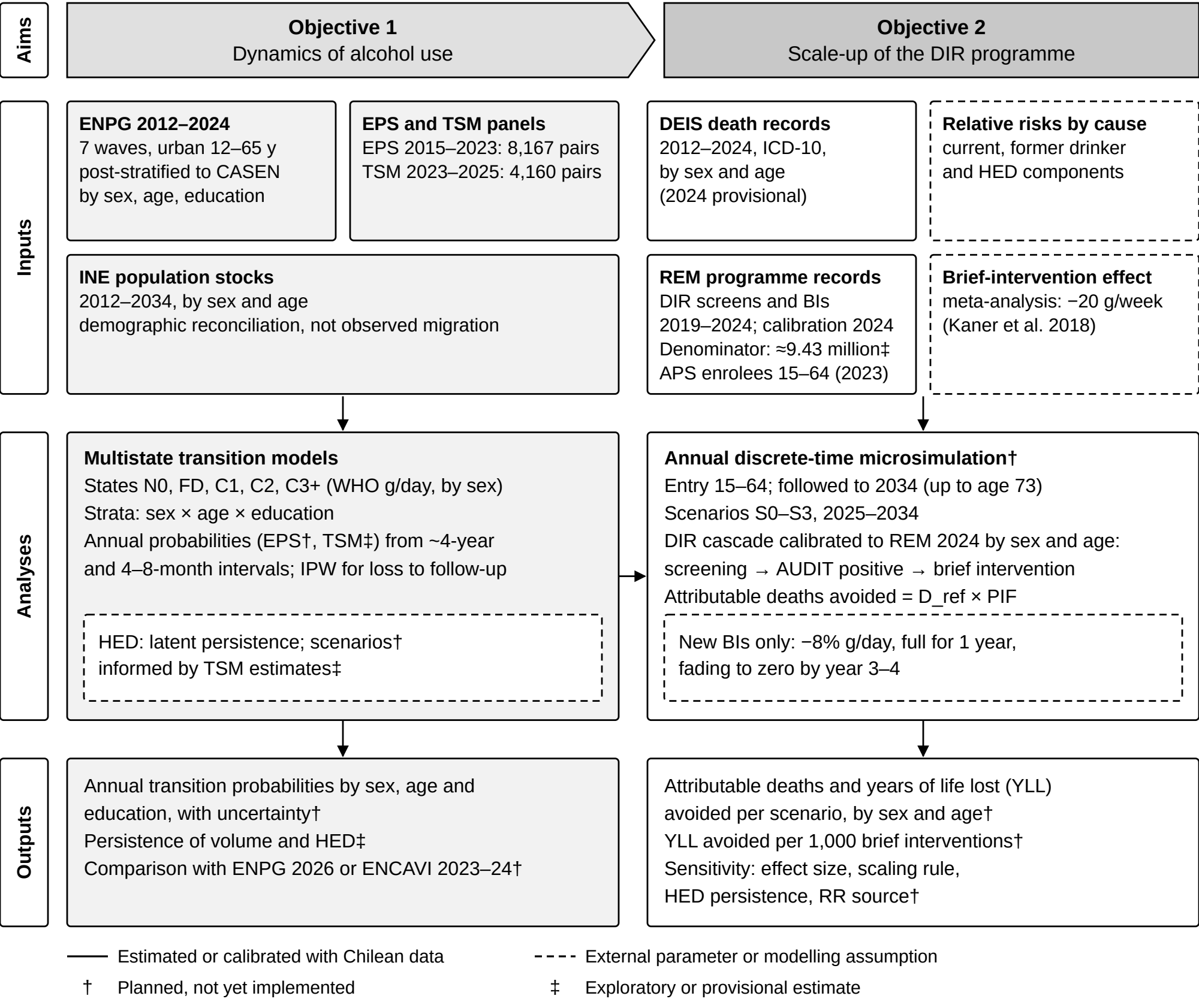
Aims
Objective 1
Dynamics of alcohol use
Objective 2
Scale-up of the DIR programme
Inputs
ENPG 2012–2024
7 waves, urban 12–65 y
post-stratified to CASEN
by sex, age, education
EPS and TSM panels
EPS 2015–2023: 8,167 pairs
TSM 2023–2025: 4,160 pairs
DEIS death records
2012–2024, ICD-10,
by sex and age
(2024 provisional)
Relative risks by cause
current, former drinker
and HED components
INE population stocks
2012–2034, by sex and age
demographic reconciliation, not observed migration
REM programme records
DIR screens and BIs
2019–2024; calibration 2024
Denominator: ≈9.43 million‡
APS enrolees 15–64 (2023)
Brief-intervention effect
meta-analysis: −20 g/week
(Kaner et al. 2018)
Analyses
Multistate transition models
States N0, FD, C1, C2, C3+ (WHO g/day, by sex)
Strata: sex × age × education
Annual probabilities (EPS†, TSM‡) from ~4-year
and 4–8-month intervals; IPW for loss to follow-up
HED: latent persistence; scenarios†
informed by TSM estimates‡
Annual discrete-time microsimulation†
Entry 15–64; followed to 2034 (up to age 73)
Scenarios S0–S3, 2025–2034
DIR cascade calibrated to REM 2024 by sex and age:
screening → AUDIT positive → brief intervention
Attributable deaths avoided = D_ref × PIF
New BIs only: −8% g/day, full for 1 year,
fading to zero by year 3–4
Outputs
Annual transition probabilities by sex, age and
education, with uncertainty†
Persistence of volume and HED‡
Comparison with ENPG 2026 or ENCAVI 2023–24†
Attributable deaths and years of life lost (YLL)
avoided per scenario, by sex and age†
YLL avoided per 1,000 brief interventions†
Sensitivity: effect size, scaling rule,
HED persistence, RR source†
Estimated or calibrated with Chilean data
External parameter or modelling assumption
† Planned, not yet implemented
‡ Exploratory or provisional estimate